

CASE STUDY

50,000 patients, 26 locations, and no way to measure how any of them were performing.

Healthcare organizations collect enormous amounts of operational data. Patient visits, provider activity, prescription records, appointment outcomes. Most of that data sits in EHR systems and databases that were built for compliance and clinical documentation, not for answering the questions that operations leaders actually need answered.

Swope Health knew their data existed. What they didn't have was a way to see it in a form that drove decisions. Armely built that layer, using the tools Swope Health already owned.

Who is Swope Health?

Swope Health Services is a Federally Qualified Health Center (FQHC) with more than 50 years of service in the Kansas City region. They serve over 50,000 patients across 26+ physical locations, offering primary care, dental, behavioral health, and pharmacy services. As an FQHC, Swope Health operates under Uniform Data System (UDS) reporting requirements, which add a compliance dimension to every data initiative.

50K+

Patients Served
Across 26+ locations

6

Dashboard Dimensions
Provider, facility, admin, Rx,
appointments, service lines

0

New Software Required
Built on Swope's existing stack

The Problem

Swope Health had grown rapidly. More patients, more locations, more providers, and more complexity. But the reporting infrastructure had not kept pace with the growth.

The core challenge was measurement. Leadership needed to understand productivity at three levels simultaneously: how individual providers were performing, how each facility was functioning, and how service lines were trending. None of those views existed in a form that was current, consistent, or comparable across locations.

Administrative staff effectiveness was entirely unmeasured. The clinical side had at least some visibility through the EHR. The operational side had almost none.

COVID had disrupted everything, but the organization had no analytical framework to understand how, or to compare the pre-COVID baseline against current performance. Every comparison required a manual data pull that was outdated before it was finished.

Prescription compliance added another dimension. Understanding whether providers were prescribing medications appropriately required a view of prescribing behavior by provider that didn't exist in any current report.

Appointment outcomes, no-shows, cancellations, reschedules, were tracked at a surface level but not in a way that surfaced trends by facility, provider, or service line.

Signs your healthcare data isn't working for you:

- Provider productivity is compared across locations using spreadsheets built by hand
- Administrative performance is measured informally or not at all
- Appointment no-show rates take hours to pull and are stale by the time they arrive
- UDS reporting is a multi-day manual process each reporting cycle
- You can't quickly answer whether a specific facility is improving or declining

The Approach

Swope Health already owned the tools: eCW for electronic health records, SQL Server for the data warehouse, BridgeIT for data integration, and Tableau for visualization. Armely's role was not to replace that infrastructure. It was to connect it properly and build the reporting layer on top of it.

This approach shortened the implementation timeline and eliminated the need for additional software licensing or staff retraining. Swope Health's staff already knew Tableau. The dashboards they received felt familiar from day one.

Data Integration

Armely sourced data directly from the SQL Server data warehouse using SQL scripting, bypassing the need for additional BridgeIT toolboxes. This reduced integration complexity and kept the data pipeline direct and maintainable.

Dashboard Development

Six dashboard dimensions were built, each addressing a specific operational question leadership had been unable to answer consistently.

- **Provider Productivity:** output by provider, comparable across facilities and service lines, with date-range filtering for period-over-period comparison.
- **Facility Effectiveness:** throughput and efficiency metrics by location, making it possible to identify which facilities were trending up or down and why.
- **Service Line Performance:** volume and productivity by service type, surfacing which lines were growing and which were underperforming.
- **Administrative Staff Visibility:** operational metrics for the non-clinical team, making administrative performance measurable alongside clinical performance for the first time.
- **Prescription Compliance:** prescribing behavior by provider, flagging patterns that deviated from appropriate prescribing criteria.
- **Appointment Outcomes:** no-show, cancellation, and reschedule trends by provider, facility, and time period.

Pre- and Post-COVID Comparison

Built-in period comparison logic let leadership pull any metric across the pre-COVID baseline and the post-COVID environment side by side. This was not a separate report. It was a filter built into every dashboard that already existed.

UDS Compliance

UDS data rules were incorporated into the dashboard logic, so the reports Swope Health used for operational decisions were built on the same data definitions required for federal reporting. This reduced the manual work of reconciling operational reports with compliance submissions.

Before	After
Provider productivity tracked separately, no comparison across locations	Unified provider productivity dashboard across all facilities
Administrative staff effectiveness not measured	Admin productivity surfaced alongside clinical metrics
Pre/post-COVID comparison required manual data pulls	Built-in period comparison with date-range filtering across all views
Prescription compliance checked manually	Appropriate vs. inappropriate prescribing tracked by provider in real time
Appointment trends required custom report each time	No-show, cancellation, reschedule trends visible and filterable
Reports slow, stale, and built in isolation	Live Tableau dashboards connected directly to the SQL Server data warehouse

What Changed

Swope Health's leadership team can now answer the questions that drive operational decisions without waiting for a manual report. Provider productivity is comparable across facilities. Facility performance is visible in real time. Administrative effectiveness is measured. Appointment trends surface patterns that were previously invisible.

The dashboards did not require new software, new licenses, or retraining staff on unfamiliar tools. They required connecting what already existed and building the reporting logic that turned raw EHR data into operational intelligence.

That distinction matters for healthcare organizations. The data is usually there. The question is whether it's organized in a way that anyone can use.

"The implementation formed a strong core for utilizing data as a decision-making tool that Swope continues to leverage and build upon."

— Armely, LLC

About Armely

Armely is a Dallas-based Microsoft solutions partner. Founded in 2017, we serve over 50 clients across energy, healthcare, government, and manufacturing. Our services cover Microsoft Dynamics 365, Microsoft Fabric, Power Platform, data analytics, and AI implementation. We

work as an extension of your team, from initial strategy through deployment and ongoing support.

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